جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : حنان رمضان عبدالرازق محمد
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** حلوان
رقم البطاقة : 28012142403742 **رقم التليفون :** 01117510993 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 14-12-1980 **المهنة :** حاصل علي دبلوم

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الإجابات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: حنان رمضان عبدالرازق محمد
National ID: 28012142403742

GUSTAVE ROUSSY

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : ---:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
حنان
رمضان عبدالرازق محمد
١٥ ش كامل ابوصالح-الملاءه
حلوان - القاهره
٢٨٠ ١٢ ١٤ ٢٤ ٠٣٧ ٤٢
JD4279097


٢٨٠٢٢١٤٢٤٠٣٧٤٢
٢٠٢١/١١
دبلوم تجاره
انثى مسلمة متزوجة
عاطف سعد خليفه احمد
البطاقة سارية حتى ٢٠٢٨/١١/٢٨

رئيسه

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

05/10/2026

MDT FORM

Name:	[illegible]	MRN:	9964
Age:	45 Y	ID:	28012142403742
Menopausal Sta	VAG BLEEDING????	Phone:	01117510993
NRS SCORE:	PS 0	Marital Status:	MARRIED
BMI:	20	Family History:	-VE
CO Morbidity:	OCP, DM	Reason Of MDT:	NEW CASE

Clinical Data:

BX: mixed IDC & ILC

ER: 0\0 PR: 0\0 HER2: -VE KI67: 80%

Sonomammography:
ACR C
LT BREAST LIQ , THICKENED SKIN AT 7 O CLOCK WITH IPSILATERAL SUSPICIOUS AXILLARY LNs

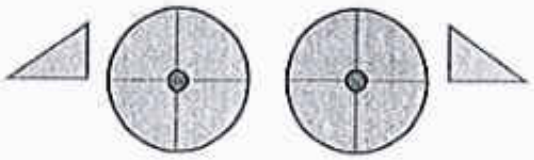
Mets Workup:
CT CAP >> RT ADENEXAL CYST ,, UTERINE LESION
BONE SCAN >> FREE

History:
PT COMPLAINS OF VAGINAL BLEEDING 5 MONTHS AGO
EXAMINATION >>> T4B , N1

Recommendations:
CESM
D&C(OUTSIDE) AND MRI PELVIS REQUESTED
PT FOR NACT THEN MASTECTOMY

MDT Outcome & Plan
NACT TAXOL CARBO PEMBRO X 4 CYCLES THEN PEMBRO AC THEN MASTECTOMY
waiting for MRI PA
D&C RESULT
CA125

R L

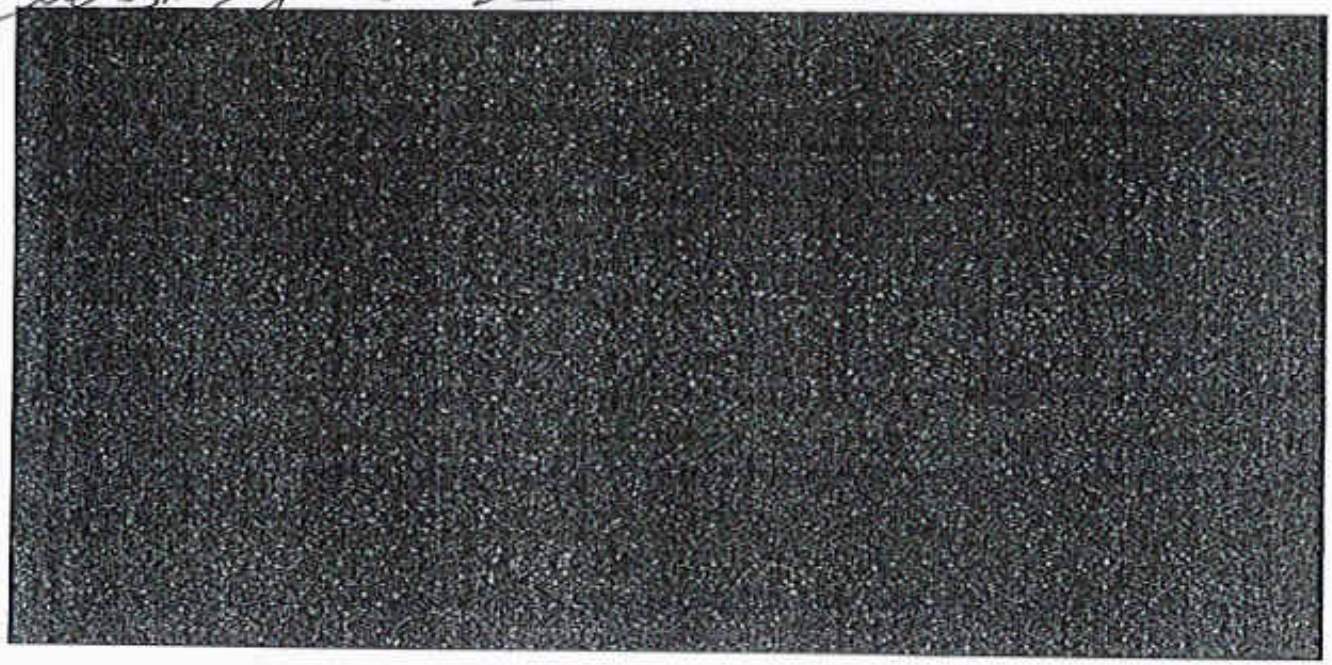


TNM

17/0/17

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



MDT Form
[[DOCTYPE.MDT_FORM]]

Name: حنان رمضان عبدالرزاق محمد MR. : 9964	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : تاريخ وساعة الخروج :

وبعد الكشف والفحوصات والمتابعة وجد انه

5/2026 mdt :(triple negative breast cancer): NACT TAXOL CARBO PEMBRO X 4 CYCLES THEN PEMBRO AC THEN MASTECTOMY waiting for MRI PA D&C RESULT CA125 BRCA TEST

prem left breast cancer ct4b n3 mo cts rt adnexal cyst uterine fibroid bs free pt did d&c disordered proliferative endometrium idc ilc mixed grade 3 triple negative ki 80% for w1 taxol 140 carbopl auc 2 c1 keytruda 200 echo pending wt 71 ht 160 sa 1.77 . on exam left breast edema redness peau d orange left ax ln 2x3

vaginal bleeding has stopped after D&C, ECHO 28/6 65% today for w2 TC next week pharmacy w3 5/7 clinic 12/7 after labs TC + C3 KEYTRUDA

on exam left breast edema decreased left ax ln 2x3 still vaginal bleeding for trial of zoladex kara for mdt bec dand c disoerdered prolif endometrium for c2 keytruda 200 w4 taxol 140 carbopl auc 2 w5 ,6 ph thyroid profile next time

for c3 keytruda + w7 taxol carboplatin w8 and w9 pharmacy neck US > supraclav path LNs .. pt initially dx as cT4b N1 M0.. CESM showed supraclav LN which were not included in the MDT .. actually stage is cT4b N3 M0, for bc from LN

pt reports episodes of fever (39 degrees celsius) that last for 1 day post cycle for w8 TC + c2 keytruda if availabe + gastric supp tx, w9 pharmacy

Pharmacy Visit For C2 Keytruda W9 Taxol /Carbo auc2

(((brca1 2 no mutation)))) neck us left pathological subcentimetric supraclav ln true cut biopsy pending cc fever after each cycle mostly hsv reaction grade 2 pn for w10 taxol 140 carbopl auc 2 with perfalgan and solucortef w11 ph on exam left breast edema induration decreaded left ax ln 1x2 left supraclav 1x2

clinical pharmacy note : w11 taxol 140 mg carbo auc 2 and solucortife in pre medication

تحريرا في ,

التاريخ : 19-09-2026 الوقت :

الطبيب المعالج : dina makram

يعتمد ,

[[DOCTYPE:OTHER_DETAILED_MEDICAL_REPORT]]

Name : Hanan Ramadan Abdelrazek Mohammed
Date : 20-04-2026 09:57
Branch : Dokki
Medical No. : 354830
Ref.Physician Prof : Goustav Roussey Egypt
Ins.Company : تعاقد مستشفيات

Tc-99m MDP ISOTOPIC BONE SCAN

Clinical History: Female patient, 45 years old; History of breast carcinoma. Evaluation for possible skeletal metastases.

Technique: Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time.

Findings:

- The skeletal system demonstrates physiological radiotracer distribution with normal uptake in the axial and appendicular skeleton.
- No focal abnormal areas of increased or decreased tracer uptake are identified to suggest skeletal metastatic deposits on scintigraphic basis.
- Expected age-related degenerative changes may be seen (at different levels of the spine and around big joints) without suspicious metastatic pattern.

IMPRESSION:

- No scintigraphic evidence of skeletal metastases on whole-body bone scan.
- Follow-up imaging only if clinically indicated.

Much obliged,
Dr. Alaa Nagi, MD
Nuclear Medicine

Bone Scan
[DOCTYPE:BONE_SCAN]

جوستاف روسي

٩٩٦٤

Patient's Name:	**Hanan ramadan abd elrazik**
Date:	**2026-04-22**
Referred by:	Prof.Dr. cap+c
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA. no available previous films for comparison.

I- CHEST:

- Left breast lesion is seen measuring 24 x 19 mm associated with skin thickening for sono-mammography.
- Multiple left axillary and subpectoral lymph nodes largest measuring
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- Umbilical omentocele.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Right adnexal cyst is seen measuring 22 mm in diameter.
- Suspected small uterine lesion measuring 21 x 16 mm.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric para-aortic and bilateral external iliac lymph nodes.

CT Scan
[[DICOMTYPE:CT_SCAN]]

Patient's Name:	**Hanan ramadan abd elrazik**
Date:	**2026-04-22**
Referred by:	Prof.Dr. cap+c
Hospital:	**Goustave Roussey International Egypt**

- No ascites.

Opinion:

- **Left breast lesion with axillary lymphadenopathy for sono-mammography.**
- **Right adnexal cyst ? functional for follow up.**
- **Suspected uterine lesion likely fibroid for sonographic assessment.**

Dr.sherif Wagdy

Dr.sherif Wagdy

CT Scan
[[DOCTYPE.CT_SCAN]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / حنان رمضان عبدالرازق محمد	Request No.	: 13005307
Patient No.	: 28012142403742	Request Date	: 04/03/2026
Path No.	: 619/26	Verification Date	: 11/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 45 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast LIQ mass (BIRADS 5), diagnosed mixed invasive duct & invasive lobular carcinoma grade 3.
Nature of Specimen: Tru-cut biopsy, collected on 4/3/2026, at 11:00 am. Cold ischemia time is 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
- IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 80%.

Conclusion:
Left breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 80%.

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[DOCTYPE&IC]

Patient Name	الأستاذه / حنان رمضان عبدالرازق محمد	Request No.	13005307
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IMMUNOHISTOCHEMISTRY REPORT

and 5. between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.

Verified by :
Dr.Duaa Mohamed

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Immunohistochemistry
[DOCTYPE&IG]

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Pathology Report

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Gross Examination:
- Fragmented grey white tissue cores, ranging in size between 0.2 cm & 1.3 cm long, totally submitted.

Microscopic Examination:
* Histological type: Mixed invasive duct and invasive lobular carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Lobular carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angioinvasion: Absent.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Mixed invasive duct and invasive lobular carcinoma, Grade 3.
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahim Ghonaim.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name:	Hanan Ramdan Achrazek Mohamed		
Patient Id:	2801214 2403742	Sex:	F
Study:		Age:	45Y 2M
Date:	28/02/2026	Accession:	
Dear Prof. Dr.		*Thank you for referring this case.*	

Bilateral breast sono-mammography revealed:-

- **Mammography:**
- **ACR: c**
- **Left breast** showing lower inner quadrant ill-defined lesion with overlying skin thickening is seen
- **By US:** showing ill-defined area seen at 7 o'clock showing heterogeneaty with posterior acoustic shadowing & overlying edematous skin & ipsilateral suspicious axillary LNS where thickened peripherally located cortex, measuring up to 7.3mm
- **Rt breast** showing diffuse homogenous density. showing No evidence of suspicious micro calcification, No evidence of solid masses nor densities
- **By US:**
- **Rt breast is** showing diffuse echogenic mottled breast parenchyma with no evidence of cysts nor solid masses seen
- Rt axilla showing multiple reactive LNs with preserved fatty hila & some of them showing accentuated fatty hila & thick cortex measuring up to 5mm

Conclusion:

- Left breast suspicious malignant mass with ipsilateral suspicious axillary LNs & localized ?? inflammatory carcinomatosis (BIRADs V) for clinical correlation & histopathological assessment & MRI breast

Dr/ Nashwa Elsayed.Msc

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	: الأستاذه / حنان رمضان عبدالرازق محمد	Request No.	: 13005307
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- Ki67%: 80%.

Conclusion:
Left breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
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* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

Verified by :
Dr.Duaa Mohamed

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Immunohistochemistry
[DOCTYPE:IHC]
CamScanner

GUSTAVE ROUSSY
INTERNATIONAL

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Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

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For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahim Ghonaim.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
CamScanner

GUSTAVE ROUSSY
INTERNATIONAL

NAME: hanan ramadan abelrazek
DATE:2/8 /2026

Neck ultrasound examination

Findings

Thyroid gland

- Left supraclavicular few sub-centimetric LNS, the largest is measuring 5x6mm likely pathological
- Bilateral cervical multilevel sub-centimetric LNS likely non specific
- Average size thyroid lobes& isthmus showing homogenous texture, normal vascularity, no gross nodules
- Normal movement with swallowing
- Central trachea
- Intact carotid vessels
- **Opinion**
- ***Left supraclavicular few subcentimetric LNS likely pathological***

Best regards
Dr. NERMEEN RAMADAN FEBR, MD

Ultrasound
@DOCTYPE ULTRASOUND
CamScanner

Patient Name	الأستاذه / حنان رمضان عبدالرازق محمد	Request No.	13005307
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Age	45 Y	Department	Out Patient
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		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

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Immunohistochemistry [DOCTYPE:IHC]
CamScanner

Patient Name:	Hanan Ramdan A.ehrazek Mohamed		
Patient Id:	2801214 2403742	**Sex:**	F
Study:		**Age:**	45Y 2M
Date:	28/02/2026	**Accession:**	
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- **ACR: c**
- **Left breast** showing lower inner quadrant ill-defined lesion with overlying skin thickening is seen
- **By US:** showing ill-defined area seen at 7 o'clock showing heterogeneity with posterior acoustic shadowing & overlying edematous skin & ipsilateral suspicious axillary LNS where thickened peripherally located cortex, measuring up to 7.3mm
- **Rt breast** showing diffuse homogenous density. showing No evidence of suspicious micro calcification, No evidence of solid masses nor densities
- **By US:**
- **Rt breast is** showing diffuse echogenic mottled breast parenchyma with no evidence of cysts nor solid masses seen
- Rt axilla showing multiple reactive LNs with preserved fatty hila & some of them showing accentuated fatty hila & thick cortex measuring up to 5mm

Conclusion:

- Left breast suspicious malignant mass with ipsilateral suspicious axillary LNs & localized ?? inflammatory carcinomatosis (BIRADs V) for clinical correlation & histopathological assessment & MRI breast

Dr/ Nashwa Elsayed.Msc

Mammogram
@DOCTYPE MAMMOGRAM
CamScanner

اسم الفرع حلوان 2
مدير الفرع د.دينا محمود محمد قنديل
مدير الوحده ا.د رانيا درويش
طبيب الوحده ا. د. علياء الدش

رئيس مجلس الادارة
ا. د. مؤمنه كامل

مدير المعامل
ا. د. هند الشربيني

PATIENT NAME			Registered	23-07-2026 21:36:59	Collected	23-07-2026 21:39:19
حنان رمضان عبد الرازق			Authenticated	12-08-2026 17:41:32	Printed	25-08-2026 18:51:18
Visit Number	Age	Gender	Referred By		Client ID	
72326507202	46 Year	Female	Prof : -		55452	

MOLECULAR BIOLOGY REPORT

BRCA 1.2 WHOLE GENE SEQUANCING / BLOOD (Astrazneca)

Result : No Pathogenic variants detected in BRCA1 or BRCA2 genes sequencing.

Background

- Hereditary Breast Cancer accounts for about 5-10% of female breast cancer and 4-40% of male breast cancer. The most common cause is mutations in the BRCA1 & BRCA2 genes which are tumor suppressor genes located on chromosomes 17 and 13 respectively.

- Pathogenic variants of these genes are inherited in an autosomal dominant manner and are associated with hereditary breast and ovarian cancer (HBOC) syndrome.
- HBOC syndrome increases the risk for breast (female and male), ovarian, fallopian tube, peritoneal, pancreatic, and prostate cancers. Additionally, BRCA2 carriers may be at increased risk for melanoma.

Method

- Genomic DNA was extracted from the patient's specimen. Libraries were prepared according to manufacturer's protocol then next-generation sequencing was performed on the Illumina MiSeq platform using CE-IVD BRaCA assay

- The assay analyzes sequencing of all exons and immediately adjacent intronic regions of BRCA1 & BRCA2 genes.

- Raw sequence data analysis, including base calling, demultiplexing, alignment to the hg19 human reference genome (Genome Reference Consortium GRCh37), and variant calling is done using standalone software. Variants are interpreted using publicly available databases such as Exac, GnomAD, dbSNP, ClinVar, Brca exchange. Variants are classified according to the American College of Medical Genetics and Genomics (ACMG) guidelines.

- Benign and likely benign variants are not included in this report.

- Variants of unknown significance classification may change over time.

Limitations

- Large rearrangements may not be detected, variants in regions with high GC content or pseudogenes may give false negative or false positive results.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

19014
almokhtabar.com
CAP ACCREDITED
حب نفسك
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PATIENT NAME			Registered	23-07-2026 21:36:59	Collected	23-07-2026 21:39:19
حنان رمضان عبد الرازق			Authenticated	12-08-2026 17:41:32	Printed	25-08-2026 18:51:18
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72326507202	46 Year	Female	Prof : -		55452	

- The test result is for clinical reference and clinicians should interpret it in the context of the patient's condition, clinical findings, family history, other lab investigations, and international guidelines for treatment.

References

- Yoshida, Reiko. "Hereditary breast and ovarian cancer (HBOC): Review of its molecular characteristics, screening, treatment, and prognosis." Breast Cancer 28.6 (2021): 1167-1180.

- Richards, Sue, et al. "Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology." Genetics in Medicine 17.5 (2015): 405-423.

Rania Darwish

Dr. Rania Darwish
Prof of Clinical & Chemical Pathology,
Faculty of Medicine, Cairo University

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جوستاف روسي

٩٩٦٤

Patient's Name:	**Hanan ramadan abd elrazik**
Date:	**2026-04-22**
Referred by:	Prof.Dr. cap+c
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA. no available previous films for comparison.

I- CHEST:

- Left breast lesion is seen measuring 24 x 19 mm associated with skin thickening for sono-mammography.
- Multiple left axillary and subpectoral lymph nodes largest measuring
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- Umbilical omentocele.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Right adnexal cyst is seen measuring 22 mm in diameter.
- Suspected small uterine lesion measuring 21 x 16 mm.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric para-aortic and bilateral external iliac lymph nodes.

CT Scan
[DOCTYPE(CT_SCAN)]
CamScanner

Name : Hanan Ramadan Abdelrazek Mohammed	Date : 20-04-2026 09:57
Branch : Dokki	Medical No. : 354830
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

DIAGNOSTIC CENTER

Tc-99m MDP ISOTOPIC BONE SCAN

Clinical History: Female patient, 45 years old; History of breast carcinoma. Evaluation for possible skeletal metastases.

Technique: Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time.

Findings:

- The skeletal system demonstrates physiological radiotracer distribution with normal uptake in the axial and appendicular skeleton.
- No focal abnormal areas of increased or decreased tracer uptake are identified to suggest skeletal metastatic deposits on scintigraphic basis.
- Expected age-related degenerative changes may be seen (at different levels of the spine and around big joints) without suspicious metastatic pattern.

IMPRESSION:

- No scintigraphic evidence of skeletal metastases on whole-body bone scan.
- Follow-up imaging only if clinically indicated.

Much obliged,
Dr. Alaa Nagi, MD
Nuclear Medicine

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Patient Name	: الأستاذه / حنان رمضان عبدالرازق محمد	Request No.	: 13005307
Patient No.	: 28012142403742	Request Date	: 04/03/2026
Path No.	: 619/26	Verification Date	: 11/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 45 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast LIQ mass (BIRADS 5), diagnosed mixed invasive duct & invasive lobular carcinoma grade 3.
Nature of Specimen: Tru-cut biopsy, collected on 4/3/2026, at 11:00 am. Cold ischemia time is 20 seconds.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Negative nuclear reaction in tumor cells (score 0/8)
- PR: Negative nuclear reaction in tumor cells (score 0/8)
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 80%.

Conclusion:
Left breast mass, Tru-cut biopsy:
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67%: 80%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

Verified by :
Dr.Duaa Mohamed

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GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذه / حنان رمضان عبدالرازق محمد	Request No.	: 13005307
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Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast LIQ mass (BIRADS 5).
Nature of Specimen: Tru-cut biopsy, collected on 4/3/2026, at 11:00 am. Cold ischemia time is 20 seconds.

Gross Examination:
- Fragmented grey white tissue cores, ranging in size between 0.2 cm & 1.3 cm long, totally submitted.

Microscopic Examination:
* Histological type: Mixed invasive duct and invasive lobular carcinoma.
* Grade 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
* Ductal carcinoma in situ: Absent.
* Lobular carcinoma in situ: Absent.
* Perineural invasion: Absent.
* Angioinvasion: Absent.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Mixed invasive duct and invasive lobular carcinoma, Grade 3.
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ibrahim Ghonaim.

Verified by :
Dr.Duaa Mohamed

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جوستاف روسي

٩٩٦٤

Patient's Name:	**Hanan ramadan abd elrazik**
Date:	**2026-04-22**
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Hospital:	**Goustave Roussey International Egypt**

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- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric para-aortic and bilateral external iliac lymph nodes.

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Patient ID: 96064	Patient Name English: Hanan Ramadan Abdalrazek
Age: ٢٥ سنة ٥ شهر	2026/06/08 11:18:01 AM Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: Pathologically proven left breast mixed lobular & ductal carcinoma, grade III, triple negative molecular subtype, KI 67 80%.

CC & MLO low energy & recombined (post-contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: c (Heterogeneously dense breast parenchyma that may obscure small lesions)

RIGHT BREAST:

- **Low energy (Non contrast) images:** show no masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images:** show no pathological mass or non-mass enhancement.
- Normal skin / nipple / areolar complex.
- **Complementary US:** Non-specific axillary lymph nodes.

LEFT BREAST:

- **Low energy (Non contrast) images:** Multiple speculated high density mass lesions seen at posterior thirds of UOQ & LIQ with wide spread architectural distortion affecting most of the breast.
- Diffuse breast edema pattern in the form of thickened skin and coarsened trabeculae.
- **Recombined (Post contrast) images** show multiple speculated heterogeneously enhancing masses of moderate conspicuity with intervening multiple areas of non-mass enhancement; ***those changes are seen involving the whole breast and reaching the nipple.***
- Largest two masses are seen at UOQ posterior third 13 cm from nipple measuring 23 x 22 x 21 mm and LIQ , 8 cm from nipple measuring 26 x 26 x 36 mm ***(Index lesion).***
- **Complementary ultrasound:** Multiple pathologically proven malignant masses, largest at LIQ ***(index)*** reaching the skin with intervening altered Non-mass pathological parenchyma involving the whole breast and reaching the nipple.
- Diffuse breast edema pattern in the form of thickened skin reaching 8 mm, interstitial and subdermal edema and coarsened trabeculae.
- Multiple (more than three) pathological level 1, 2, and 3 axillary nodes are noted. Two small left supra-clavicular nodes are noted, largest measured 6 x 5 mm.

IMPRESSION:

RIGHT BREAST: (BIRADS 1= normal) No pathological mass or non-mass enhancement.

LEFT BREAST: (BIRADS 6= pathologically proven malignant) Multi-centric locally advanced pathologically proven malignant mass and non-mass enhancement along with ipsilateral pathological axillary and supra-clavicular lymph nodes.

Best regards,

Dr. Amal Hegazi, M.Sc.

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GUSTAVE ROUSSY
Gustave Roussy International-Egypt Pathology Lab
جوستاف روسي الثرلتيونال - مصر معمل الباثولوجي
جوستاف روسي

Patient Name	حنان رمضان	**Case No.**	361754
Sex/Age	Female 45 Y	**Patient No.**	26554
Registered Date	31/8/2026 04:16:43 PM	**Referred By Dr.**	Interventional Radiology
Validation Date	13/9/2026 01:10:46 PM	**Branch**	Gustave Roussy
Reporting Date	13/9/2026 01:58:03 PM	**Pathology No.**	2789/26

Pathology Report

Clinical Data: Left Pathological Infraclavicular Lymph Node After Left Mastectomy

Nature of specimen: Tru-Cut Biopsy, collected on 31/08/2026, at 02:00 pm. Cold ischemia time is 20 seconds.

Gross Examination
- Multiple fragmented greyish white tissue cores ranging in size from 0.2 to 1.0 cm, totally submitted.

Microscopic Examination
- Histopathological examination revealed normal nodal tissue with patent subcapsular sinuses. The lymphoid follicles are hyperactive and have prominent germinal centers. There is no evidence of specific granuloma or neoplasia in sections examined.

Diagnosis:
Left Infraclavicular Lymph Node; Tru-Cut Biopsy:
- Consistent with reactive lymphoid hyperplasia.
- No metastatic deposits.

Signature
Dr. Walaa Ibrahim Ghonaim, MD.

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GUSTAVE ROUSSY
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي الكرنشوتال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	حنان رمضان	**Case No.**	**361754**
Sex/Age	**Female 45 Y**	**Patient No.**	**26554**
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Validation Date	**13/9/2026 01:10:46 PM**	**Branch**	**Gustave Roussy**
Reporting Date	**13/9/2026 01:58:03 PM**	**Pathology No.**	**2789/26**

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Signature
Dr. Walaa Ibrahim Ghonaim, MD.

CAP
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Professor Dr. Emad El-Din Roshdy
Abdel Maqsoud
Bachelor of Medicine and Surgery
PhD in Pathology - Professor of Pathology
Faculty of Medicine, Al-Azhar University

معمل الأستاذ الدكتور / عماد الدين رشدى
عبدالمقصود
تحاليل الأنسجة والأورام
بكالوريوس الطب والجراحة
دكتوراه الباثولوجى – أستاذ الباثولوجى
كلية الطب جامعة الأزهر

PATHOLOGY REPORT

Name: HANAN RAMDAN ABDELRAZIK

Date: 23-5-2026

Age: 45

Referred by: Prof.Dr: MOHAMMED FATHEY

CLINICAL DATA; FEMALE, 45 D&C BIOPSY WAS DONE FOR HYSTOPATHOLOGICAL EXAMINATION.

Gross examination: single container with abundant curetting.

Microscopic Examination; sections showed endometrial tissue composed of endometrial glands and stroma, the glands were small, rounded lined by single layer of cubical cells with eosinophilic cytoplasm rounded, central basal nuclei. The cells showed bland cytological and nuclear features. The glands were unevenly distributed. The stroma was fragmented and hemorrhagic. Foci of endocervical tissue composed of endocervical mucous secreting glands lined by tall columnar cells with bland cytological features admixed with stroma composed of spindle shape cells with bland cytological and nuclear features.

DIAGNOSIS: D&C BIOPSY; DISORDERD PROLIFERATIVE TYPE ENDOMETRUIM.

NO ENDOMERITIS, NO HYPERPLASIA AND NO MALIGNANCY.

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وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

Oncology Center مركز الأورام
خطابات أورام

الاسم : حنان رمضان عبد الرازق

التاريخ ٢٠٢٦/١٠/٠١

MRI PELVIS:

- Average size anteverted uterus showing thickened endometrium & uterine cervix region 9.5mm for clinical correlation.
- No pelvic masses or collections.
- Normal appearance of the rectum and ischiorectal fossa on both sides.
- Normal urinary bladder.
- Normal pelvic fat.
- Otherwise pelvic bones appear normal.

Prof. Dr. Sheriff Abdel-Fatah

Hala

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351 Korniesh El Nile Aghakhan Shoubra Cairo Tel. 22039164 22039356 Fax 22039 236